

TREATMENT OF ECLAMPSIA

Initial care

- **Lateral decubitus** position, shielded from light
- **Guédel's cannula** in the mouth + upper airway suction
- Placement of a **venous line** Placement
- of an indwelling **urinary catheter**
- **Sampling** :
 - Blood type
 - CBC+ Platelets
 - Blood ionogram, uricemia, creatinine, transaminases
 - Coagulation assessment: Cephalin Kaolin Time (TCK), Fibrinogen Degradation Products (PDF).
- **Continuous monitoring of vitals** (Monitoring sheet): every 5 min. then every 15 min.
 - Pulse+ Bdcf+ BP
 - Osteotendinous reflexes
 - Diuresis
 - Respiratory rhythm

Drug treatment

- **Oxygen**: 6 litres/min
- **Anti-convulsants** :
 - **Magnesium sulfate** :
 1. Loading dose: **4 g** (8ml at 50% Magnesium Sulfate + 12ml EPPI or isotonic saline) IVL over 20 min+ **5 g** (10ml at 50%) IM in each buttock with 1 ml lidocaine 2%.
 2. Maintenance dose: **5g** (10ml at 50%) IM every 4h alternately in the buttocks or **1g** IVL every hour
 3. Continue magnesium sulfate up to 24 hours after delivery or after the last seizure.
 - **If magnesium sulfate not available, Diazepam** (valium): 1 amp of 10mg *diluted in 10ml* IVDL, then infusion of 40mg in 500ml Ringer Lactate or Isotonic Saline to be infused over 6 to 8 hours
- **Caution**: depressant effect of diazepam on the fetus
- **Fluid and electrolyte resuscitation**
 - 1litre/8h **Isotonic Saline** or **Ringer Lactate**
- **Antihypertensives**: **hydralazine** 5-10 mg slow IV every 5 minutes until BP drops, then repeat every hour if necessary, or give 12.5 mg IM every 2 hours as needed. **Nifedipine** ½ cp of 10mg sublingually, to be repeated after 10mn if no improvement.
- **Referral to a SONUC center**

Obstetrical treatment: Deliver within 12 hours maximum

WORK IN PROGRESS

NO WORK

LAG PHASE

ACTIVE PHASE

CESARIENNE

Fetal distress: Caesarean section

No fetal distress: assist expulsion (cupping)

+ REANIMATION of the NEWBORN

- **Appreciate** :
 - signs of fetal distress
 - local conditions collar condition, presentation, etc.
- **Discuss how to proceed**:
 - Caesarean section
 - a labor trigger
 - The goal is to deliver within 12 hours

* PREPARATION OF THE DOSE **REQUIRED** WITH MAGNESIUM SULFATE :

- 1ml of **50%** magnesium sulfate solution contains 0.5g $MgSO_4$, hence :
 - 10 ml contain 5g of **50%** Magnesium Sulfate
 - 4 ml contain 2g of **50%** Magnesium Sulfate
- to reduce to **20%**, = 1 fl 10ml at + 15 ml EPPI; → 25ml at **20%** → **20ml = 4g $MgSO_4$ at 20%**.

In case of Magnesium Sulfate overdose: FR < 16 /min, or diuresis < 100ml/4h or absence of osteotendinous reflexes: stop the treatment. magnesium sulfate and give antidote **1g calcium gluconate** 10ml 10% IV